A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 May 2026

Dr Sally Cockburn
Junction Place Medical Centre

Dear Dr Cockburn,

Re: Mrs Alida Reggars
DOB: 13/08/1947 Phone: 0414 606 337
Address: 304/619 Canterbury Road, SURREY HILLS, VIC, 3127
Email: lidyreggars@gmail.com

Summary

- **Resolution of recent gastrointestinal infection with persistent faecal incontinence.**
- **Focus on nutritional optimisation and weight gain prior to escalating fibre therapy.**
- **Planned referral for bowel-specific pelvic floor physiotherapy.**
- **No action required by you.**

I had the pleasure of reviewing Alida via telehealth. You will be pleased to know she is feeling significantly better following her recent gastrointestinal infection. The frequent, loose motions she was experiencing have resolved, and she is no longer reporting the six bowel movements per day she had at the height of her illness.

However, Alida continues to struggle with faecal incontinence. While her stool consistency has improved to a thickened, "Dairy Queen" consistency, she is still experiencing accidents without warning, typically in the afternoon or evening. This is a common and distressing manifestation of her underlying scleroderma.

A significant portion of our consultation focused on her nutrition. Alida has lost weight recently, and I have emphasised that regaining her weight and strength is currently our highest clinical priority. We discussed the use of Metamucil or other fibre supplements to thicken the stool and reduce the risk of accidents; however, Alida finds that these supplements cause abdominal cramping and premature satiety. I have advised her to hold off on fibre for now to ensure she can maintain an adequate caloric intake. Once her weight has stabilised, we may consider a more easily tolerated supplement such as partially hydrolysed guar gum (PHGG).

Impression and Plan

I have discussed several strategies to manage her incontinence. One option we explored is the use of a morning glycerin suppository to ensure the rectum is empty at the start of the day, which can often mitigate the risk of accidents later on. Furthermore, I believe she would benefit significantly from bowel-specific pelvic floor physiotherapy and biofeedback. I am currently looking into local options for her, though I have mentioned Cherry Desert Cripigney and Angela Khera as highly experienced specialists in the gastrointestinal manifestations of scleroderma.

Alida recently received her rituximab. I have asked her to contact me immediately if she experiences a recurrence of gut symptoms or further weight loss, as I do not want her to "tough out" these symptoms.

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Otherwise, I have planned to review her in six months' time.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Laura Ross

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